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Voluntary Knowledge Brokering to Promote Evidence-Based Nursing Practice: A Qualitative Study

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ABSTRACT

Knowledge brokering is a process of connecting knowledge producers with users to facilitate evidence-based practice through relationship building and information sharing. This descriptive qualitative study aimed to clarify knowledge brokering by nurses in Japanese hospitals. Twelve registered nurses in Japanese hospitals participated. They had over 5 years' clinical experience, including experience in conducting research, particularly staff research, and education. Data were collected through semi-structured individual interviews and analyzed using qualitative content analysis. The analysis revealed a central theme: continuous efforts to foster empathy among colleagues and spontaneously promote evidence-based practice: multifaceted brokering activities by clinical nurses. Findings identified 10 categories categorized into four interconnected gears: establishing the foundational ground, assessing clinical needs and staff readiness, tailoring and diffusing evidence, and sustaining and evolving evidence-based practice. Even nurses without formal titles voluntarily bridged the research-practice gap, providing new insights into informal brokering. Brokers communicated considerately, balanced evidence with clinical context, negotiated practical compromises, and fostered staff research competency.

1 | Background

Nurses are expected to make evidence-based, accurate judgments and provide care that responds appropriately and promptly to clinical situations. However, the integration of research evidence into practice often takes an average of 17 years (Morris et al. 2011). Evidence-based practices (EBP) can involve a single piece of evidence or multiple complex guidelines, making implementation challenging (Harrison and Graham 2021). In Japan, limited time and opportunities to engage with prior research, along with entrenched habits, hinder the adoption of EBP. No formal evaluation of Japanese nurses' EBP competencies or structured EBP education programs has been established (Tomotaki and Tsuda 2021), despite the importance of applying research findings in practice.

Healthcare organizations employ knowledge translation strategies to promote EBP and improve the quality of care (Harrison and Graham 2021). Knowledge brokering stands out as a key type of knowledge translation that helps share and apply the best evidence in clinical practice (Catallo 2015; Dobbins et al. 2019; Minian et al. 2021). It connects knowledge producers (researchers) with knowledge users (healthcare providers) to move evidence into action (Bullock et al. 2016).

However, few studies have examined how organizational knowledge brokers influence knowledge brokering in hospitals (Glegg and Hoens 2016). Furthermore, the role is often informal and inconsistent, complicating assessments of its effectiveness (Newman et al. 2020). Clarifying the activities of nurse knowledge brokers in hospitals is essential to advancing EBP.

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Key Points

- Even without formal titles, nurses in Japanese hospitals voluntarily bridge the research-practice gap through "steady efforts to broaden empathy" and trust-building.
- Brokers facilitate evidence-based practice not just by transferring knowledge, but also by negotiating practical compromises in challenging clinical situations.
- As informal leaders, these knowledge brokers actively develop staff competency by translating evidence and fostering research education within their teams.

Implementation science is a key strategy for integrating EBP into clinical practice. In this field, CFIR 2.0 (Damschroder et al. 2022) helps explain what makes it difficult or easy to put new ideas into practice by focusing on "relational connections" and "communications" in organizations. This includes personal skills, teamwork, and connections with other organizations. The i-PARIHS framework (Harvey and Kitson 2016) highlights "facilitation" as key to making changes. This idea is similar to the roles of healthcare knowledge brokers, who manage information, connect people, build skills, help out, and evaluate. Both frameworks show that changing clinical practice needs strong relationships and active support. Recent evidence further demonstrates that formal implementation lead nurses require active facilitation, collaboration, and organizational support to successfully drive EBP (Aljuaid and Al-Moteri 2024). While these frameworks show exactly what is needed for implementation, how staff nurses actually build these relationships in their daily work remains unclear.

This knowledge gap exists because while these models emphasize the importance of facilitation skills, implementation is often led by designated internal or external facilitators, or others with specific organizational influence. However, in Japan, even Certified Nurse Specialists (CNSs), who are advanced practice nurses, face significant barriers to EBP such as a lack of organizational readiness and time constraints (Tomotaki et al. 2023). Therefore, practicing EBP is naturally even more difficult for general clinical nurses. Nurses with a strong interest in EBP still critically require continuous learning opportunities and institutional support to sustain their commitment to EBP (Tomotaki et al. 2024). Recent studies highlight that nurses continue to face substantial educational, organizational, and knowledge-related barriers to implementing EBP (Lai et al. 2024). Furthermore, successful EBP implementation depends not only on individual motivation but also heavily on organizational capacity, information literacy, and access to evidence (Halili et al. 2025). Given these realities, understanding how some nurses overcome resource constraints by voluntarily acting as informal knowledge brokers becomes highly relevant. Even with limited support, some nurses voluntarily share evidence with their team.

Although prior research examined the roles of advanced practice nurses (Gerrish et al. 2011) and middle managers (Boutcher et al. 2022a, 2022b), and clarified the knowledge brokering process of Japanese master's-prepared nurses (Akasaki et al. 2021), further research is needed to clarify how a broader range of clinical nurses act as informal brokers and integrate these activities into their busy daily practice. Knowledge brokering was developed from the understanding that passive dissemination alone is insufficient to influence knowledge uptake or clinician behavior. Instead, effective knowledge mobilization relies on a combination of approaches, including making connections through relationships, disseminating synthesized knowledge, and facilitating interactive learning (Ward 2017).

Nurses typically prefer asking their colleagues for clinical information rather than reading written guidelines (Fossum et al. 2022), a trend also clear in Japan (Futami et al. 2019). Therefore, colleagues who can act as informal knowledge brokers and share reliable evidence can play a very important role in advancing EBP. While active relationships enhance the effectiveness of knowledge brokers, establishing such engagement presents serious communication and interpersonal challenges (Yamanie et al. 2023). Furthermore, a systematic review of knowledge brokers in health-related settings by Cross et al. (2023) noted that little is known about the specific characteristics of these brokers and how they connect with peers. Therefore, qualitative research is needed to elucidate the micro-level, interpersonal adjustments of clinical nurses who possess at least 5 years of experience and are involved in hospital research or education. This study clarifies how they voluntarily build relationships and implement EBP.

2 | Aims

This study aims to clarify how clinical nurses in Japanese hospitals build relationships with staff and engage in voluntary knowledge-brokering activities to promote EBP.

3 | Methods

3.1 | Design

This study employed a descriptive, qualitative design with semi-structured interviews. Such an approach is appropriate when gathering information directly from those who have experienced a phenomenon (Bradshaw et al. 2017) or when a straightforward description is desired (Sandelowski 2000). Qualitative descriptive research allows for rich accounts of phenomena that are not well understood (Bradshaw et al. 2017). Given the limited research on knowledge brokers in Japan, this design was deemed suitable. The study followed the Consolidated Criteria for Reporting Qualitative Research guidelines (Tong et al. 2007), as it provides a detailed framework for reporting qualitative interviews.

3.2 | Study Setting and Recruitment

The participants were 12 registered nurses working in Japanese hospitals. The participants were nurses with at least five years of clinical experience, prior research experience, and extensive experience in staff research and education. Recruitment combined purposive and snowball sampling (Ritchie et al. 2013). A research request was mailed to three

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nurses meeting the inclusion criteria. Those willing to participate returned a postcard with their availability and email address. The researcher coordinated the interview details (time, location, etc.) via email. After each interview, participants were asked to recommend additional study participants, and research requests were sent to those individuals. This process yielded nine more participants. All interviews were conducted face-to-face in a private room to ensure confidentiality.

3.3 | Data Collection

Data were collected from April to November 2020 through individual, semi-structured interviews. The interview guide, developed from prior studies, was discussed and finalized by the authors. No pilot testing was conducted due to the small sample size. Participants were provided in advance with an operational definition of knowledge brokering adapted from Canadian Health Services Research Foundation (CHSRF 2003). It was defined as connecting people to build relationships, identifying their needs, and sharing ideas and evidence with the goal of using the best evidence to improve their work. They were then asked to recall relevant experiences. The interview guide (Table 1) included questions about how they engaged in knowledge brokering.

The first author conducted all interviews, which were audio-recorded with consent. Notes were taken during each session. Interviews lasted 49–114 min (average: 79 min). Each participant was interviewed once.

3.4 | Data Analysis

This study employed qualitative content analysis (Graneheim and Lundman 2004; Graneheim et al. 2017). All interviews were transcribed verbatim, producing transcripts 3–40 pages long. The first author read transcripts repeatedly, extracted text related to knowledge brokering, and developed 1601 meaning units. These were condensed and coded ($n = 110$). Codes were classified into 29 subcategories, which were further organized into 10 categories. Coding was revised through discussions among the authors until consensus was reached. Data sufficiency was reached after the tenth interview and saturation after the twelfth, when no new themes emerged (Fusch and Ness 2015). Examples of coding and categorization are shown in Table 2.

TABLE 1 | Interview guide.

No.	Interview question
1	In what situations, for whom, and how did you engage in knowledge brokering?
2	What attitude or stance did you take when engaging in knowledge brokering?
3	What did you pay attention to when engaging in knowledge brokering?
4	What sort of ideas did you use to carry out knowledge brokering?
5	How did you deal with challenges and worries while carrying out knowledge brokering activities?
6	Is there any knowledge or ability that you think is necessary to engage in knowledge brokering?
7	What changes have you noticed in your practice or target audience since engaging in knowledge brokering?

3.5 | Ethical Considerations

Ethics approval was obtained from the research ethics committee of the author's academic institution (application number: 2019-59). Participants received verbal and written explanations of the study's significance, purpose, methods, confidentiality measures, and voluntary nature. Informed consent was obtained from all participants, who received no financial or other incentives.

3.6 | Rigor and Reflexivity

Reliability in qualitative descriptive research was addressed using the four principles outlined by Lincoln and Guba (1985): credibility, confirmability, dependability, and transferability. Before the interviews, the first author established rapport with the participants. During the interviews, empathy and active listening were used to build trust. The first author conducted the interviews and used the same interview guide to ask all participants the same core questions to enhance dependability. Research positionality was clarified: the first author had conducted prior research on knowledge brokers during a master's program, and the co-author had long-standing experience in nursing education and research (Barrett et al. 2020). Both are women with interdisciplinary backgrounds. The analysis results were member-checked by the nine participants, who cooperated in June 2021. After reviewing a summary table of categories and subcategories, they provided feedback that was incorporated to improve accuracy. Participants had 7–34 years of experience, represented nine hospitals, and worked across various departments.

Confirmability of the results was ensured by including direct quotes from nurses with diverse experience. Purposive sampling and detailed participant descriptions support transferability. The categorization process was reviewed by two experts involved in graduate education for specialist nurses, documented in an audit trail, revised by the first author, and finalized with consensus.

4 | Results

4.1 | Characteristics of Participants

The participants included nine women and three men, with an average of 17.5 years of nursing experience (range: 7–34 years), representing a broad range of expertise. Among them were three CNSs, three Certified Nurses (CNs), and four nurse administrators. The Japanese Nursing Association (2023) recognizes the CNS and CN certifications as denoting high levels of expertise, where CNSs assume six core roles (expert practice,

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TABLE 2 | Example coding tree for the "Curating and Generating Evidence " ategory.

Meaning unit	Code	Subcategories	Categories
"Seeing is believing when I see something that concerns me or something I think could be improved. I offer advice to the patient at the bedside on how to improve their condition and invite the staff to join in the care." (D-103)	Implement evidence-based care with staff	Using evidence with staff	Curating and Generating Evidence
"Basically, nursing is possible because of the ward nurses, so I try not to exclude them and work together with them to provide care. Even when I am doing something myself, I ask the ward staff to stay with me so that I can say, 'Please take care of the next.'" (G-45)			
"There is something called a family assessment tool. For example, we use the tool to score whether a family member has a high need for information or medical security, and then visualize and graph the results. As such, the staff can share information about the kind of care that needs to be provided." (J-53)	Adoption of evidence-based tools		
"We made a ruler so that it would not be a burden to the staff. There is something called the Bristol scale, and it is very effective to ask questions like this. If you turn the ruler over, it becomes a pain scale. In other words, we made it easy for the staff to use it in the clinical setting." (B-161)			
"I told my staff that if they were going to spend the same amount of effort, it would be better to provide effective care rather than continue with the same (useless) care as before." (E-90)	Communicating evidence updates		
"(Even if the older staff have been doing something habitually, the basis for providing that care may be outdated.) The younger staff members learn by watching the older staff members, and they do the same things as the older staff members. However, we sometimes tell the younger staff members that (what the older staff members are doing)it is no longer being done." (K-91)			
"I think if you present evidence that is beneficial to patients, everyone says, 'I see,' and they are pretty supportive" (D-98).	Explanation of the intention to introduce evidence	Communicate creatively	
"People forget things if you just tell them the results. There has to be a reason. I try to explain why it seems like this is the case, or how it is." (K-125)			
"It is easier to understand something by seeing it with your eyes than by conveying it in words. If you are going to convey something in words, it is easier to visualize numbers and share values." (I-84)	Communicate visually to staff		
"Many of our staff members are not good at reading, so I make it easier for them to read by telling them, 'Try reading this,' or by choosing books with lots of pictures." (J-96)			

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TABLE 3 | Participants' sociodemographic characteristics.

Participants	Sex	Main department of affiliation	Years of nursing experience	Position	Qualifications
A	Female	Outpatient department	20–29	Nurse Administrators	RN, CN in Cancer Chemotherapy Nursing
B	Female	Administration department	30–39	Nurse Administrators	RN, CNS in Cancer Nursing
C	Female	Medical and surgical ward	10–19	Staff	RN, CNS in Gerontological Nursing
D	Female	Medical and surgical ward	10–19	Staff	RN, CN in Breast Cancer Nursing
E	Male	ICU	10–19	Staff	RN
F	Female	Psychiatric ward	20–29	Nurse Administrators	RN
G	Female	Administration department	5–9	Staff	RN, CNS in Wound, Ostomy and Continence Nursing
H	Female	Administration department	20–29	Nurse Administrators	RN
I	Male	Psychiatric ward	10–19	Staff	RN
J	Female	Medical and surgical ward	10–19	Staff	RN
K	Female	Medical and surgical ward	5–9	Staff	RN
L	Male	ICU	5–9	Staff	RN, CNS in Critical Care Nursing

Abbreviations: CN, Certified Nurse; CNS, Certified Nurse Specialist; ICU, Intensive Care Unit; RN, Registered Nurse.

consultation, coordination, ethical coordination, education, and research) and CNs focus on three (advanced practice, guidance, and consultation). While some participants held professional or managerial positions that involved these roles, none had been given a dedicated position, time, or organizational mandate to implement EBP. As most participants were unfamiliar with the academic term "knowledge broker," it was defined alongside specific examples during the interviews. Nevertheless, participants recognized that their brokering activities were based not on formal directives but on a voluntary and personal professional conviction to provide the best possible care in their daily nursing practice. Sociodemographic characteristics are presented in Table 3.

4.2 | Knowledge Brokering by Nurses

Our analysis revealed a central theme: "Continuous efforts to foster empathy among colleagues and spontaneously promote EBP: Multifaceted knowledge brokering mechanisms employed by clinical nurses." The 10 categories are structured as a central driving force and four interconnected gears (Figure 1). The core category, the strong desire to provide the "best nursing practices," acts as the driving force. This force powers the four gears formed by the remaining categories. This metaphor illustrates that, in knowledge brokering, all gears operate simultaneously, creating a fluid dynamic that drives EBP. (Note: "staff" refers to fellow registered nurses throughout the remaining text).

4.3 | Categories

4.3.1 | The Driving Force: Striving for Best Practice

Brokers wanted to provide the best care, improve their units, and help staff grow. This strong professional conviction served as the internal engine for all their voluntary brokering activities. They consistently reflected on their own actions through communication, learned from teaching peers, and asked for advice when they reached the limits of their expertise:

> I want nurses to find nursing more interesting, which motivates me to provide better care.
>
> (K-151)

Brokers sought to ensure consistency in care by building a unified care framework based on conversations with patients. They taught staff to see the true meaning behind patients' actions. Instead of forcing textbook rules, they used evidence within realistic limits, always prioritizing the patients' wishes and individuality:

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FIGURE 1 | Multifaceted knowledge brokering mechanisms by clinical nurses.

> Although the nursing plan is subdivided, it is important to value the will, thoughts, and individuality of the patient. [...] I think it can be called individuality. [...] I think that evidence is really the scientific part, and it needs to be reflected in the subdivided care plan and in actions.
>
> (B-76)

4.4 | Gear 1: Establishing the Foundational Ground

4.4.1 | Fostering Empathetic Connections

Brokers viewed relationship-building as the key to sharing knowledge. They regularly communicated to build friendly relationships, recognizing that if staff "can easily ask questions... [they] will be able to work more efficiently and it will be better for the patients" (F-31). Brokers kept this communication open by carefully hiding their busyness or negative emotions:

> If I say, 'I'm sorry, but I can't talk to you right now,' it will be difficult for the other person to say anything, so no matter how busy I am or outside of work hours, I try to help people who need it.
>
> (C-63)

They acted as reliable role models while treating all staff as equals, ensuring that colleagues never perceived any hierarchical distance:

> When the assistant head nurse (who occupied a more senior position than me) comes to me for advice, it is hard to say that we are on an equal footing, but I try to treat her as a professional colleague, respecting her expertise as a certified skin and excretion care nurse, regardless of hierarchical standing.
>
> (G-10)

During consultations, brokers carefully read staff members' emotions to understand their true needs, routinely considering "whether they just want me to listen, or whether they want an answer" (A-242). By respecting staff's values without denial, brokers created a safe space for knowledge exchange. Finally, they motivated peers through small, daily gestures of appreciation:

> Like saying, 'The way you talked to them earlier was really great,' or 'Thank you for coming to help.' I think receiving these small statements of appreciation is what's truly important.
>
> (L-45)

4.4.2 | Bridging and Networking

Brokers expanded their alliances by identifying key individuals and mapping peer relationships to serve as bridges. They consistently developed a shared understanding with ward staff before consulting specialists:

> I thought that if I went with the certified nurse, the head nurse would be more willing to listen. So, I went with the certified nurse. I told the head nurse, 'I think this is something that all the nurses should think about together. Could you please talk to the liaison staff (psychiatric nursing specialist nurses) together with us?' The head nurse said, 'Okay, let's do that.' Together, we got everyone involved.
>
> (K-114)

Beyond their unit, they daily communicated with multidisciplinary teams to exchange patient information. They also proactively connected doctors with nurses by collecting and proposing nursing evidence:

> I gathered nursing evidence from various sources, such as nursing magazines and presented it to the internal medicine doctor.
>
> (B-91)

4.5 | Gear 2: Assessing Clinical Needs and Staff Readiness

4.5.1 | Identifying True Clinical Needs

Brokers always questioned the current situation in their organization, triggering reviews of unit manuals and reconsideration

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of the evidence behind routine care. By paying close attention to staff comments and actions, brokers identified underlying educational needs and understood broader organizational issues. Most importantly, brokers did not limit themselves to staff members' daily troubles or their simple desire to help; instead, they used these insights to identify what patients truly needed:

> The staff's needs were, simply put, that the patients were in pain, and they wanted to alleviate that pain. When I say pain, I don't mean that the patients themselves were saying it, but that their eyes were actually open and they couldn't close them because of eyelid edema. Many staff members felt that if they had to do that, they would feel pain. So, there was a need for how to alleviate or improve that pain, and that's what we approached.
>
> (E-15)

4.5.2 | Assessing Staff Readiness and Organizational Culture

Brokers assessed the readiness of staff to implement EBPs. They carefully observed each staff member's interests, weaknesses, personalities, and values. They also assessed their mindset simply by listening to their self-reported perceptions of their patients. This approach helped brokers judge whether a nurse was ready to adapt evidence to patients:

> What suits a first-year staff member differs completely from a 10-year staff member. I think experience is very important.
>
> (H-77)

Furthermore, brokers considered the factors that facilitated or hindered the use of evidence. They observed the shared values and overall culture of the organization to determine whether evidence could easily take root. Brokers actively analyzed why staff found being aware of evidence difficult and why new practices failed to become robust in the workplace:

> [After I explained the evidence on nutritional supplements], I felt like it hadn't really taken root. Like, I told them, but nothing changed. I think that because the staff themselves hadn't experienced whether the patients responded well or badly [after using the evidence], they ended up choosing the easier option for themselves. I think that this isn't just a problem with that staff member, but with the organization as a whole department. So, I think we should have done more to influence that.
>
> (D-134)

4.6 | Gear 3: Tailoring and Diffusing Evidence

4.6.1 | Curating and Generating Evidence

Brokers used the internet to collect reliable evidence, including the latest guidelines and articles from influential journals. They critically assessed this literature to ensure high quality and maintained a broad perspective across their interpretations of various sources. Then, they considered if these findings applied to their organization:

> I reviewed meta-analyses and RCTs, then assessed if the content fit my facility. Also, there were guidelines for nurses who treat sepsis, and eye care was mentioned in those guidelines, so I looked into that.
>
> (E-29)

When existing evidence was insufficient, brokers conducted their own research to meet true clinical needs. Aiming to reduce patient suffering and staff burden, they gained cooperation from multidisciplinary professionals and interested peers:

> Patients with spreading skin ulcers (from breast cancer) are difficult to manage. It takes time and effort for the staff to treat them, and patients don't like it because it's painful and distressing. [...] we did a little research on how to make treatment easier and less painful for both parties. [...] we started with a core group of very interested people, so I think the fact that we weren't alone was a strength.
>
> (D-55)

4.6.2 | Translating and Weaving Evidence Into Practice

Brokers synchronized evidence with their own practical knowledge. They timed their proposals carefully to avoid sounding pushy, weaving evidence into casual small talk or gentle suggestions. They secured staff understanding by clearly explaining the purpose and reasons for relying on particular pieces of evidence:

> Teaching experienced nurses how to use medicines is key, but explaining the reasons helps sustain learning.
>
> (C-133)

They also shared evidence based on clinical experience. Rather than preaching abstract theory, they demonstrated evidence through hands-on care, working side-by-side with peers:

> If you explain it from a theory perspective, it often doesn't sink in. [...] So, I think it's easier to get people to understand if it actually works well when you try it out, and then you tell them that there is evidence.
>
> (K-80)

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Recognizing different staff capabilities, brokers creatively tailored their communication. They translated complex data into plain language using visual aids, highlighted literature, brief memos, and standardized assessment tools. For example, they recommended easy-to-understand books:

> I would print books with many pictures and encourage reading, knowing many staff avoid dense texts. That's the kind of ingenuity we used.
>
> (J-96)

Brokers avoided sounding pushy by providing specific evidence that linked directly to daily care. They strongly emphasized evidence related to patient safety. They also informed staff when previously used evidence changed, encouraging them to search for evidence independently:

> A lot of people warm up enemas... but recent papers say you don't need to anymore. So I told them... But I also say, 'Don't assume everything I say is correct; please look it up yourself too.' [...] I think it's difficult unless they are interested in finding out why it's no longer necessary.
>
> (F-80)

Finally, brokers created environments to share evidence. They carefully planned study sessions by selecting focused topics and adjusting schedules so everyone could easily attend. They shared information on seminars and created opportunities for staff to voice their opinions at conferences:

> We hold study sessions at set times and ask staff to communicate evidence learned in meetings.
>
> (G-88)

4.7 | Gear 4: Sustaining and Evolving EBP Practice

4.7.1 | Evaluating Impact and Closing the Feedback Loop

First, brokers carefully checked how staff received their ideas. They measured staff understanding by listening to casual remarks, reading medical records, and reviewing questionnaires. They also encouraged staff to evaluate their own care.

Furthermore, brokers captured the reactions of patients and their families. They verified whether the new evidence and their own practice improved the patients' health. Finally, brokers shared these patient reactions with the staff as feedback, thereby closing the loop and helping staff realize the true value of the new care:

> After trying a new approach, I checked the chart and confirmed healing was progressing well. When I can go, I go, but when I can't, I check the chart later
>
> (B-30).

4.7.2 | Anchoring Evidence Through Trial and Compromise

Brokers did not force perfect evidence; instead, they reconciled evidence with what was possible in practice. By closely observing staff reactions and unit practices, brokers thoughtfully considered alternative methods. When textbook evidence was difficult to apply, they actively sought realistic compromises:

> If you follow the textbook, method A is definitely the best. However, (considering the patient and situation), it is not possible. Another method, B, is not good, but I will try it because it is closer to method A. In fact, we are often told that it is impossible because we are busy. Especially with delirium care. When we have to do detailed, basic things, and the staff says, 'No, it is impossible,' we have to say, 'Well, can you do it if you do it like this?' When they deny it too strongly, I think that I should back off for a while. Rather than backing off, I think that this is good, but if it is not possible, let's think of another method.
>
> (C-70)

Brokers worked patiently with staff to ensure care continued. They shared information across staff members and teams so evidence would take root. They repeatedly suggested ideas until staff could carry out tasks independently, practicing together through trial and error. Finally, brokers embedded evidence into the system. They kept records of their assessments and practices, created care plans reflecting the evidence, and included this evidence directly in their units' manuals and guidelines.

4.7.3 | Cultivating Critical Thinking and Clinical Autonomy

Brokers aimed to help staff become independent practitioners rather than simply giving them answers. They supported staff in interpreting literature and taught them about types of papers and how to conduct literature reviews, emphasizing the need to carefully check data sources:

> Some nurses bring in abstracts. I explain that abstracts are not original research; staff need to examine primary documents properly... I try to point out such things.
>
> (A-73)

Brokers encouraged staff to think deeply by engaging closely with their thoughts and carefully untangling them. To help staff verbalize their actions and analyze root causes rather than relying on hearsay, brokers constantly asked "why":

> You have to hear the circumstances, you know. For example, if you hear from someone else that 'this is what happened,' I think it's a bit wrong to just believe it and leave it at that. So I try to ask the person, for

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> example, 'I heard they fell. You were with them, right? Can you tell me how it happened?'
>
> (F-146)

Brokers also explained the intent of their own actions to give staff new assessment perspectives. They used metaphors to help staff empathize with patients. They explained the importance of developing ideas independently to the staff. When correcting mistakes, brokers carefully considered staff members' feelings and chose approaches tailored to them.

To support long-term staff growth, brokers drew out the staff's interests, abilities, and what they had already learned. To encourage independent growth, brokers carefully avoided helping too much. Instead, they highly praised any actions that showed independent learning:

> The act of researching was a sign of growth, and the fact that she did something on her own led to my evaluation. I thought, 'This child has changed a little.' [...] As nurses, I think it's our job to bring that out.
>
> (J-163)

5 | Discussion

In Japan, hospital wards often lack dedicated advanced practice nurses (APNs) and formal EBP education (Tomotaki and Tsuda 2021). Thus, nurses without formal broker titles spontaneously engaged in knowledge brokering. While middle managers and APNs typically act as brokers (Burgess and Currie 2013; Gerrish et al. 2011), participants in this study included general ward nurses alongside CNSs, certified nurses, and managers. They shared the goal of carrying out "best nursing practices," aligning with EBP's emphasis on incorporating patient values (International Council of Nurses 2012). Although prior research clarified the knowledge brokering process of Japanese master's-prepared nurses (Akasaki et al. 2021), this study highlights that clinical nurses, regardless of their formal roles, use this shared goal as the driving force to turn their brokering gears. A recent review identified three universal broker roles: linkage agents, knowledge managers, and capacity builders (Cross et al. 2023). Our findings show how the four interconnected gears, powered by this driving force, put these roles into action through "steady efforts to broaden empathy."

First, as "linkage agents" (driven by Gears 1 and 2), these nurses built trust and networks (CHSRF 2003; Oldham and McLean 2006). Lacking formal authority, empathetic trust-building was their strongest tool. They treated staff equally and intuitively understood their true feelings through direct communication. This direct approach is important, especially since impersonal methods like webinars often fail to create meaningful change (Sarkies et al. 2021). Because EBP implementation relies heavily on networks and culture (Li et al. 2018), brokers bridged professions, navigating relationships and respecting workplace culture without arrogance (CHSRF, 2003).

Second, as "knowledge managers" (driven by Gear 3 and parts of Gear 4), brokers assessed staff readiness for new evidence (Gerrish et al. 2011). Established EBP frameworks, such as the IOWA model, emphasize assessing readiness and integrating evidence (IOWA Hospitals and Clinics Office of Nursing Research, EBP and Quality [IOWA] 2018). Furthermore, while a recent paper proposed "formalized mentorship" to simplify complex evidence (Marfo et al. 2026), our study shows a different reality. Without formal mentors, brokers could not force evidence on staff. Instead, facing resistance, they sought mutually acceptable "compromises." This real-world compromise uniquely connects ideal forms of evidence with difficult clinical realities.

Finally, as "capacity builders" (Gear 4's ultimate goal), brokers supported education, which is fundamental to knowledge translation (King et al. 2024; Ward et al. 2009). Adapting to staff learning needs (Conklin et al. 2013), informal brokers avoided just giving answers. Just as mentors build critical appraisal skills (Marfo et al. 2026), they also foster critical thinking through daily conversations. While APNs facilitate research access (Bornbaum et al. 2015; Gerrish et al. 2011), our study newly highlights that informal brokers actively support staff's research activities, empowering them to practice EBP independently.

Established frameworks (e.g., CFIR, i-PARIHS, and IOWA model) recognize the importance of facilitation, relational connections, and informal networks. However, little is known about how clinical nurses actually navigate these practices in everyday clinical settings. Recent evidence shows that even formal implementation lead nurses require active collaboration and organizational support to succeed (Aljuaid and Al-Moteri 2024). This provides an important bridge to our findings: in clinical contexts where essential EBP resources and education are lacking (Halili et al. 2025; Lai et al. 2024), clinical nurses—including those in administrative positions—voluntarily fill the gap. This strengthens the theoretical justification for examining informal brokers alongside formal roles, thereby revealing concrete ways nurses actualize EBP. Thus, it adds a new perspective: brokering is not just information transfer but a human process built on "empathy" and "compromise." This informal brokering acts as a crucial "lubricant" that drives successful EBP implementation in everyday clinical settings.

5.1 | Limitations of the Work

This study has several limitations. First, the findings are based on Japanese healthcare facilities and therefore may not be directly transferable to other healthcare systems with different organizational structures and cultures. Second, although interviews were conducted in Japanese and carefully translated into English, some linguistic and cultural nuances may have been lost during translation.

A further limitation is that the interviews were conducted in 2020. Since then, healthcare systems have undergone substantial changes, particularly following the COVID-19 pandemic, which may have influenced evidence-based practice and organizational support for knowledge translation. Nevertheless,

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the focus of this study was not on time-sensitive organizational policies or technologies, but on the interpersonal processes through which experienced clinical nurses voluntarily build relationships, establish trust, and facilitate evidence use in everyday practice. These relational and facilitative processes remain consistent with the core characteristics of knowledge brokering identified in more recent evidence, including the systematic review by Cross et al. (2023), suggesting that the conceptual insights generated by this study remain relevant despite the time elapsed since data collection. Future research should examine how these informal knowledge-brokering activities have evolved in post-pandemic healthcare settings.

5.2 | Recommendations for Further Research

Future studies should explore perceptions of knowledge brokering by nurses and multidisciplinary medical professionals who interact with brokers. Developing a knowledge broker activity scale could facilitate self-evaluation. Clarifying outcomes of knowledge brokering will aid evaluation of its effectiveness.

5.3 | Relevance for Clinical Practice

In the Japanese clinical context, knowledge brokering is often a voluntary activity driven by nurses' "steady efforts to broaden empathy" and build trust among staff. To effectively promote EBP, healthcare organizations must identify and support these informal knowledge brokers who connect research with practice. Specifically, educational support that focuses on the practical activities identified in this study—such as facilitating evidence transmission and negotiating compromises—can enhance nursing staff's EBP capabilities. Recognizing and valuing these voluntary contributions is essential for the successful integration of EBP into clinical nursing.

6 | Conclusion

This study revealed that nurses without formal knowledge broker roles also voluntarily engage in knowledge brokering within healthcare facilities, offering new insights into grassroots EBP promotion. Key findings demonstrate that informal brokers communicate considerately with staff; carefully manage evidence integration while attentively observing context; seek compromises in challenging situations; and support capacity development by educating staff about research activities. These findings enhance understanding of informal brokers' roles and can guide the development of educational programs and strategies to promote evidence-based practice.

Author Contributions

Fumi Akasaki: writing – original draft, conceptualization, methodology, investigation, funding acquisition, data curation. **Yasuko Hosoda:** supervision, writing – review and editing, conceptualization, methodology, data curation, resources, writing – original draft.

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Ethics Statement

This study was approved by the Research Ethics Review Board of the Graduate School of Nursing, Osaka Prefecture University (application number: 2019-59).

Conflicts of Interest

The authors declare no conflicts of interest.

Data Availability Statement

The data that support the findings of this study are available from the corresponding author upon reasonable request.

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